



We build software for hospitals, doctors and pharmacists. Each product starts from a specific problem in a hospital, a health authority or a medical society.

Guido Putignano, Founder and President

Taranto and Zurich, info@bioergotech.org

What we do

We are a small team in Taranto and Zurich. We work with hospitals, health authorities and medical societies on a problem they already have, often first as a paid consulting project.

When a solution works, we turn it into a product that other institutions can buy. We started in April 2025.

Four products so far

Frankura

For oncology departments. Pilot running with ASL VCO.

VIS Pharma Compass

For hospital pharmacies and regions. Tested on data from Regione Abruzzo.

Bonnie

For rheumatology patients, designed with the Italian College of Rheumatologists.

Skippy

For people leaving prison, with the University of Maryland, Baltimore.

Team

Guido Putignano, Founder and President

Biomedical engineer, with experience at ETH Zurich, Harvard and MIT.

Olufemi Olusola, Technical lead

Biostatistician with 10 years of experience.

Domenica Leone, COO

Lawyer. Runs operations, legal and compliance.

Domenico Putignano, CMO

Leads the clinical side of our work.

Mario Tagarelli, Economic manager

Looks after finance, grants and reporting.

Advisors

Daniela Marotto, rheumatologist, Italian College of Rheumatologists (CRel)

Pasquale Persico, market access at Gilead Sciences

Roberto De Ponti, Managing Director, 3B Future Health Fund

Foundation board

Guido Putignano, Domenica Leone and Carmine Pisano

Who we work with

Doctors

Oncologists and rheumatologists. They lose hours to paperwork and see chronic patients only every few months.

FRANKURA AND BONNIE

Pharmacists

Hospital pharmacy directors and regional pharmacy services. They manage large drug budgets, often in spreadsheets.

VIS PHARMA COMPASS

Hospitals and regions

Local health authorities, hospitals and regional governments. They hold the budget, so they are the ones who sign the contract.

ALL PRODUCTS

Medical societies and universities are partners in some projects. The Italian College of Rheumatologists is working with us on the design of the Bonnie study, and the University of Maryland, Baltimore runs Skippy with us.

Frankura

For oncology departments. We have a pilot agreement with ASL VCO, in Piedmont, and we are running the pilot.

Every year ASL VCO handles 1,100 cancer intake forms, 525 oncology visits and 1,547 tumour board cases. Most of the data is typed by hand and copied from one form to the next.

What Frankura does

- Records the visit and fills in the intake form
- Copies the data into the tumour board file
- Prepares a summary of each case before the board meets
- Produces a standard report for GPs and other hospitals

It runs on top of the hospital's existing software.

Estimated doctor time saved per year

Filling in the intake form	 406 h
Copying to the board file	 600 h
Preparing the board	 386 h

About 1,400 hours a year, close to €67,000 at €48 an hour. That is roughly seven months of one doctor's full-time work.

Our estimate, based on ASL VCO's own volumes.

How big the market is for Frankura

\$6.9 billion

The global market for AI agents in healthcare by 2030, up from \$1.1 billion in 2025. The nearby market for AI-powered clinical documentation reaches about \$14 billion in the same year.

218 buyers

The public institutions that buy software like ours: 198 health authorities and hospital trusts in Italy, and 20 health clusters in Saudi Arabia. At €50,000 to €150,000 per licence, that is €11 to €33 million, plus a yearly fee.

20 hospitals

Our target for the next three years, worth €1 to €3 million in licences. Four Italian institutions are already talking to us: ASL Salerno, Ospedale Miulli, ASL Taranto and ASL Caserta.

Sources: MarketsandMarkets 2026, The Business Research Company 2026, CERGAS Bocconi OASI Report 2025, Health Holding Company.

VIS Pharma Compass

For hospital pharmacies and regions. A working prototype, not yet under contract.

In 2025 the Italian public health system spent €4.8 billion more than the legal ceiling on the medicines it buys directly, up from €4.0 billion in 2024. Pharmacy teams have the data but work in spreadsheets, so they spot savings late.

What VIS does

- Puts purchase and dispensing data in one place and checks it against the accounts
- Flags patents about to expire and biosimilars that are available but not used
- Compares health authorities in the same region, medicine by medicine

It only reads aggregated data, so it is not a medical device. Spending figures: AIFA, final reports for 2024 and 2025.

Tested on Regione Abruzzo's four health authorities

Over 250,000 dispensing records and about €450 million of yearly spend.

Yearly fee and estimated yearly value, for a region with four health authorities

Fee	€159,000
Value, minimum case	€404,000
Value, prudent case	€808,000

Sales start with a €9,000 analysis of the client's own data, done in three weeks.

Bonnie

For rheumatology patients, designed with the Italian College of Rheumatologists (CRel). Interface prototype ready.

A patient with rheumatoid or psoriatic arthritis sees the rheumatologist every three to six months. Flares, side effects and stopped treatments in between are rarely written down, so each visit starts from what the patient remembers.

How it works

- Every week the patient answers a few questions in about two minutes, with no account to create
- Bonnie, a small cartoon skeleton, reminds them and keeps them going. It reacts to the fact that they answered, never to what they said
- Before the visit, the rheumatologist sees how the patient has been, week by week

The study with CRel

CRel brings together about 500 rheumatologists working in hospitals and in the community. We are designing the study with them. Called PROSSIMA, it would follow patients with rheumatoid arthritis, psoriatic arthritis and axial spondyloarthritis.

CRel would own the study and the data. We build and run the software. We built the prototype at our own cost before asking them to commit.

Bonnie is built on the Frankura platform. It is not a medical device.

Skippy

For people leaving prison, with the EMBRACE Initiative at the University of Maryland, Baltimore. Interface prototype ready.

People coming out of prison are usually handed a list of phone numbers. Programmes count a referral as soon as it is sent, even if the person never gets help.

How it works

- Helps the person take the next practical step, such as getting an ID or applying for benefits
- Connects them with the right service or staff member
- Follows up until the step is done, and tells staff what got in the way

The plan with EMBRACE

Phase	People
Pilot	20
Baltimore	500
Maryland	2,000
Other programmes	later

EMBRACE owns the programme and the data. We build and run the software. It is our first project in the United States.

How we work

1. Consulting first

A hospital, region or medical society comes to us with a problem. We work on it as a paid project, or start with a paid analysis like the €9,000 one for VIS. We build a first prototype at our own cost, and the full product only once someone is paying for the work.

2. One platform

What we build goes into one shared platform, Frankura: recording clinical data, patient consent, audit trails, the AI layer and data storage in Europe. New products start from there, and Bonnie is built on it.

3. Sell it again

Once a product works for one client, we offer it to the next. ASL VCO follows the intake and tumour board model of the Piedmont oncology network, so the same workflow applies across the region. A new region for VIS needs a data adapter, not a new system.

bioERGOftech works in two parts. The Foundation finds the clinical needs and does the research and consulting. Hadi Health S.r.l., our clinical AI venture, sells and runs the software in hospitals.

Where we are in 2026

Pilot agreement

With ASL VCO for Frankura. We are running the pilot.

4 health authorities

VIS tested on real data from Regione Abruzzo.

4 in talks

ASL Salerno, Ospedale Miulli, ASL Taranto and ASL Caserta.

€25,000 signed

And about €120,000 in advanced pipeline, as of June 2026.

-  Pilot running
-  Tested on regional data
-  In talks

Map: ISTAT boundaries, CC BY 4.0.



The research behind the products

Each project is listed on bioergotech.org, with what it has shown and what it has not.

Cancer risk after inflammatory disease

Published in *Cancers*, 2026, with Daniela Marotto. A national cohort of 356,022 patients from Italian hospital discharge records. Cancer risk is highest in the first year after diagnosis.

Cancer risk by subgroup

An algorithm that links genotype, lifestyle and socioeconomic data to cancer incidence at population level. Delivered within CODIGE, part of the AGE-IT programme.

Cell-therapy models

Mathematical models of engineered cell therapies, at ETH Zurich. Review published in *Frontiers in Immunology*, 2025.

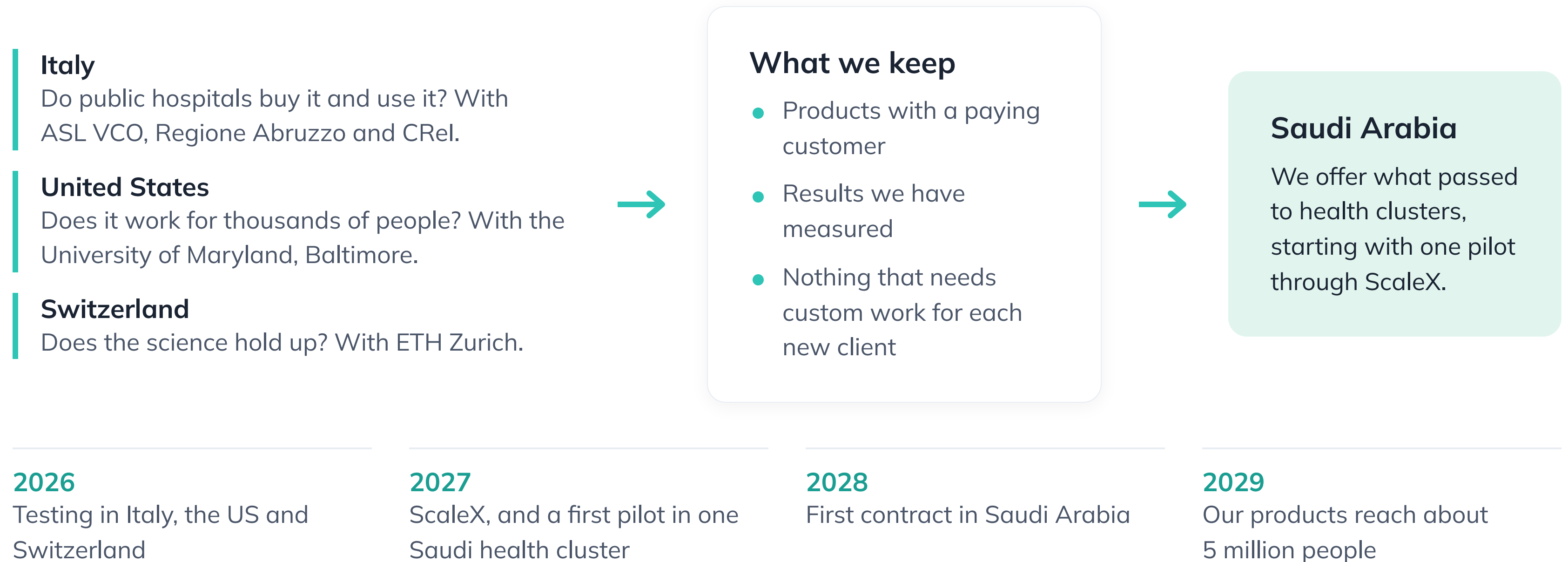
Very small patient groups

Methods for diseases too rare for standard statistics, starting with propionic acidemia in Saudi Arabia. Work in progress.

Our lab and offices are in Taranto. Guido Putignano also works at ETH Zurich's Department of Biosystems Science and Engineering.

How we get to Saudi Arabia

We test each product where it is easiest to prove, keep what works, and offer that to Saudi health clusters.



Who this reaches

5 million

Our goal for 2029: our products in use in health systems that together serve about 5 million people.

Skippy adds 2,000 people leaving prison in Maryland.

Where	People	Status
ASL VCO, Frankura	153,000	Pilot running
Regione Abruzzo, VIS	1.27 million	Tested on data
ASL Salerno, Taranto and Caserta	2.5 million	In talks
One Saudi health cluster	1 million	With ScaleX
Total	About 5 million	

Residents on 1 January 2025 (ISTAT). A Saudi health cluster serves about 1 million people (Ministry of Health), and the 20 clusters serve over 20 million (Health Holding Company).

What we are asking from ScaleX

We want to open in Riyadh with products that are already being tested in Italian public health. Saudi public healthcare is moving to 20 regional health clusters, each running the hospitals and clinics for about 1 million people. That is close to how the Italian health authorities we work with are organised.

What we would bring

- Frankura, for oncology departments
- VIS Pharma Compass, for hospital drug budgets
- Bonnie, for patients with chronic conditions between visits

What we need

- 1 An introduction to a health cluster for a first pilot
- 2 Advice on Saudi rules for clinical software and health data
- 3 Introductions to investors
- 4 Lab access at KAUST and a desk in Riyadh for our team

A first pilot would follow our Italian path

A paid analysis of the cluster's own data in three weeks, the software running within 90 days, and a joint review at six months.



Thank you

Four products, a pilot running with ASL VCO, and a plan to reach 5 million people by 2029.

Guido Putignano, Founder and President

info@bioergotech.org, bioergotech.org

Via Ciro Giovinazzi 70, Taranto, Italy